This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Aciclovir cream and tablets for the treatment of Cold Sores (Herpes Labialis)

Patient Group Direction, version 005, issued 30 September 2026. Supersedes Version 004, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE CKS Guidelines for Cold Sores

Overview

- Cold sores are caused by **Herpes Simplex Virus type 1 (HSV-1)**.
- Common, self-limiting **viral infection** of the lips and perioral area.
- May be **recurrent** after primary infection due to virus reactivation.

Symptoms

- **Prodromal symptoms**: tingling, burning, or itching
- **Painful vesicles** on lips or around the mouth
- Vesicles rupture, crust over, and heal within **7–10 days**

- May be associated with **fever, malaise, swollen lymph nodes** in primary infection

Triggers for Recurrence

- Sun exposure
- Stress
- Illness (e.g. viral infections)
- Menstruation
- Immunosuppression

Diagnosis

- **Clinical diagnosis** based on typical presentation.
- Laboratory testing (PCR or viral culture) only if:
    - Atypical presentation
    - Immunocompromised patient
    - Severe or persistent lesions

Management

Mild/Typical Cold Sores

- Usually **no treatment required**, self-limiting.
- **Topical antiviral cream** (e.g. **aciclovir 5%** or **penciclovir 1%**):
    - Apply **as early as possible** (within 48 hours of symptom onset)
    - Shortens duration by 1–2 days

Frequent or Severe Episodes

- Consider oral antivirals (e.g. aciclovir 200 mg 5 times/day for 5 days or valaciclovir 500 mg BD for 5 days). Only aciclovir 200 mg tablets are authorised under this PGD; valaciclovir is not.
- For **frequent recurrences** (≥6/year), consider **suppressive therapy**:
    - e.g. aciclovir 400 mg BD. Suppressive therapy is not authorised under this PGD: refer.

Supportive Measures

- Keep the area clean and dry
- Use lip balm or moisturiser
- Avoid kissing or close contact while lesions are present

When to Refer or Seek Specialist Advice

- Immunocompromised patients (risk of severe or systemic HSV)
- Cold sores lasting >10 days or spreading
- Severe pain or systemic symptoms
- Ophthalmic involvement (refer urgently to ophthalmology)
- Atypical, persistent, or extensive lesions

Prevention Advice

- Use sunblock lip balm if sunlight is a trigger
- Avoid sharing utensils, towels, or kissing when lesions are present
- Avoid triggers where possible

Patient Group Direction

for the supply/administration of

Aciclovir cream

for the treatment of

Cold Sores (Herpes Labialis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Aciclovir cream is indicated for the treatment of recurrent herpes labialis (cold sores) of the lips and face in adults and adolescents.
Inclusion criteria	- Adults and adolescents aged 12 years and over. - Clinical diagnosis of recurrent herpes labialis. - Informed consent obtained. - No contraindications to aciclovir.
Exclusion criteria	- Known hypersensitivity to aciclovir, valaciclovir, or any excipients. - Immunocompromised patients or severe recurrent episodes. - Lesions on mucous membranes (e.g. eyes, inside the mouth, genitals): the cream must not be applied to mucous membranes. Refer. - Pregnancy or breastfeeding. Refer to a prescriber.
Cautions	- Start treatment at the first sign of symptoms (e.g. tingling or itching). - Avoid contact with eyes or mucous membranes. - Do not share tubes to prevent cross-infection. - Counsel on hygiene measures to prevent spread of infection.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Aciclovir cream 5%
Legal category	P (Pharmacy medicine). A PGD is not legally required for a P sale; this arm is included so that the supply is assessed and recorded to the same standard as a POM supply. Where the P licence is narrower than this PGD, the P licence governs.
Storage	Store below 25°C. Do not refrigerate or freeze.
Route/method of administration	Topical application to affected area of the lips and face. Patients should wash their hands before and after applying the cream and avoid unnecessary rubbing of the lesions or touching with a towel, to avoid aggravating or transferring the infection.
Dose and frequency	Apply to affected area five times daily at approximately four-hour intervals for 5 days.
Quantity to be administered and/or supplied	Up to 1 x 2 g or 5 g tube per episode.
Maximum or minimum treatment period	Use for up to 5 days. If lesions have not healed, treatment may be continued for up to 10 days. If lesions are still present after 10 days, users should be advised to consult a doctor.
Adverse effects	Mild stinging or burning, drying or flaking of skin. Rare: erythema,

	allergic contact dermatitis. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form and route, and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Advise the person/carer to seek medical advice if their symptoms worsen (for example the lesion spreads, new lesions develop, or there is persistent fever or difficulty taking fluids), or no significant improvement is seen after 5–7 days. Reassure the person/carer that oral herpes simplex infections are usually self-limiting, and that lesions should heal without scarring. Advise on measures for symptom relief, such as: - Paracetamol and/or ibuprofen to relieve pain and fever, if required, and there are no contraindications. - Adequate fluid intake to reduce the risk of dehydration. Give advice to minimize the risk of transmission to other people and autoinoculation: - Explain that herpes simplex virus is easily transmitted to other people. - Avoid kissing and oral sex until all lesions have fully healed. - Do not share items that come into contact with lesions (for example makeup and lip balms). - Avoid touching the lesions, other than when applying topical preparations, which should be dabbed on rather than rubbed in to minimize mechanical trauma to the lesions.

	- Topical treatments should not be shared with other people. - Wash hands with soap and water immediately after touching lesions. - Take care if using contact lenses, as there is a risk of transmission to the eye if lenses become contaminated. - Defer elective dental treatment until all lesions have fully healed. - Advise the person to try to avoid trigger factors, if possible. If sunlight is a trigger, advise the use of sunscreen or sunblock lip balm (sun protection factor 15 or greater), to help reduce recurrent episodes.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Aciclovir 200mg

for the treatment of

Cold Sores (Herpes Labialis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Aciclovir tablets are indicated for the treatment of recurrent herpes labialis (cold sores) of the lips and face in adults and adolescents.
Inclusion criteria	- Adults and adolescents aged 12 years and over. - Clinical diagnosis of recurrent herpes labialis. - Informed consent obtained. - No contraindications to aciclovir.
Exclusion criteria	- Known hypersensitivity to aciclovir, valaciclovir, or any excipients. - Immunocompromised patients or severe recurrent episodes. - Lesions involving the eyes, the inside of the mouth or the genitals: outside this PGD. Refer. - Pregnancy or breastfeeding. Refer to a prescriber.
Cautions	- Start treatment at the first sign of symptoms (e.g. tingling or itching). - Counsel on hygiene measures to prevent spread of infection. - Caution is advised when administering aciclovir to patients with impaired renal function and adequate hydration should be maintained. - Elderly patients are likely to have reduced renal function and therefore caution must be considered in this group of patients too. - Tablets may be dispersed in a minimum of 50 ml of water or swallowed whole with a little water.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Aciclovir 200mg tablets
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration with or without food.
Dose and frequency	200 mg five times daily for 5 days.
Quantity to be administered and/or supplied	25 tablets per episode.
Maximum or minimum treatment period	5 days. No extension under this PGD: severe or immunocompromised cases are excluded and referred.
Adverse effects	Nausea, vomiting, diarrhoea, headache, dizziness. Rare: rash, fatigue, renal dysfunction. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form and route, and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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	- Take care if using contact lenses, as there is a risk of transmission to the eye if lenses become contaminated. - Defer elective dental treatment until all lesions have fully healed. - Advise the person to try to avoid trigger factors, if possible. If sunlight is a trigger, advise the use of sunscreen or sunblock lip balm (sun protection factor 15 or greater), to help reduce recurrent episodes.

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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v005

Issued: 30 September 2026

Supersedes: Version 004, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v004

Issued: 14 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover corrected to supply (not administration) and the arm titles corrected from 'for the treatment to' and 'for the treament to' to 'for the treatment of'.

Guidance summary now states that valaciclovir and suppressive aciclovir are not authorised under this PGD.

Exclusion rows reworded: mucous membrane involvement is stated as a patient criterion (refer) rather than a route restriction, and pregnancy or breastfeeding is a referral rather than a prescriber assessment that a PGD cannot provide.

Tablet arm treatment period is 5 days with no extension, matching the exclusion of severe and immunocompromised cases and the fixed quantity of 25 tablets.

Records row template text separated into date, dose, form, route and quantity, and the adverse reaction bullet no longer runs into 'supplied via PGD'.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 28 December 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

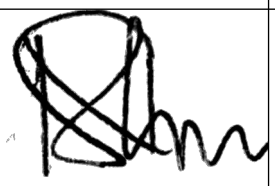
Changes in this version:

Legal category corrected: aciclovir 5% cream is a P medicine (and GSL in the 2 g pack), not a POM. The oral aciclovir arm is POM and unchanged.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v005, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.